



NAD+

Information sheet written for the 1000 mg vial TrueHold holds in stock. Not a generic internet protocol.

THIS VIAL — STARTING DOSE (read this first)

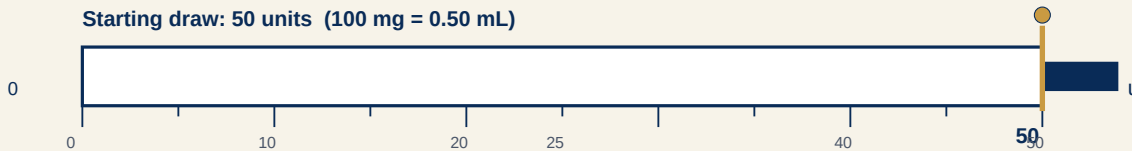
NAD+ · 1000 mg lyophilized · mix with 5 mL bacteriostatic water · 200 mg/mL · 1 unit = 2000 mcg

Start: draw **50 units** on a U-100 syringe (**0.50 mL = 100 mg**). Fill the 0.5 mL (50-unit) U-100 syringe to the 50 mark.

Route: Subcutaneous (belly or upper arm)

Rhythm: Twice per week (about every 3–4 days). Not a daily shot.

Starting draw: 50 units (100 mg = 0.50 mL)



1. What it is — the molecule, how large it is

NAD+ (nicotinamide adenine dinucleotide) is not a peptide. It is a coenzyme found in every living cell. Formula C₂₁H₂₆N₇O₁₄P₂ (oxidized form). Molecular weight 663.4 daltons — smaller than Semax. TrueHold stocks a 1000 mg lyophilized vial in a larger ~5 mL bottle, not a 2 mL peptide vial.

Mitochondria — where NAD+, MOTS-c, and SS-31 work

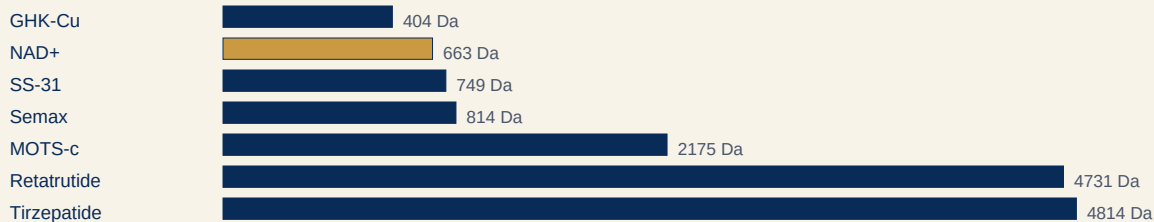


Inner membrane (cristae + cardiolipin) — SS-31 binds here

NAD+ shuttles electrons for ATP

MOTS-c is a 16-aa mitochondrial message to muscle / AMPK

How large is it? (daltons, log-ish bars — bigger bar = bigger molecule)



2. How it works in the body

NAD+ shuttles electrons in mitochondrial energy production (NAD+ ↔ NADH). It is also the fuel for sirtuins and PARP DNA-repair enzymes. When NAD+ is low, those systems run poorly. The body makes it from tryptophan and from vitamin B3 salvage (niacin, NR, NMN). An injectable vial is a direct NAD+ salt, not the same thing as an NR capsule.



3. Testing data — animal

Raising NAD⁺ in mice (usually via precursors) has been tied to mitochondrial function, DNA repair, and metabolic resilience. That literature is about biochemistry, not a license for megadose injections.

4. Testing data — human, what is expected, hopes

Oral niacin is an old FDA-approved lipid drug with flushing and liver-risk at high modified-release doses — different molecule, different route. NR and NMN oral trials exist. Intravenous NAD⁺ is used in clinics with little gold-standard RCT evidence for 'anti-aging.' TrueHold's locked protocol is subcutaneous on this 1000 mg vial: 50 units (100 mg) twice weekly to start.

- More stable daytime energy
- Recovery and training tolerance
- Focus
- Healthy-aging biochemistry (sirtuins / PARPs) — a research hope, not a proven human outcome from this vial

5. Have people hurt themselves? Risks

People feel unwell on high NAD⁺ pushes: flushing, chest tightness, nausea, cramping, anxiety during fast IV drips. Subcutaneous shots can sting. High-dose nicotinic acid (not identical to this salt) can injure the liver — another reason not to stack random B3 megadoses. There is no large safety trial of twice-weekly 100 mg subcutaneous NAD⁺ as an anti-aging drug. Stop for chest pain, severe shortness of breath, or yellowing skin.

6. FDA — approved or not

This injectable NAD⁺ vial is NOT an FDA-approved drug for energy, aging, or addiction. Niacin (nicotinic acid) tablets have FDA-approved lipid uses; that is not this product. Dietary NAD⁺ precursors are supplements, not this 1000 mg research vial.

7. Reconstitution for this exact vial

Check the label: it must say NAD⁺ and 1000 mg powder in a larger ~5 mL glass vial. You need bacteriostatic water (BAC), alcohol swabs, and U-100 insulin syringes (0.5 mL / 50-unit for every starting draw on this sheet; 1 mL / 100-unit if a later step exceeds 50 units).

- Wash hands. Wipe the rubber top with an alcohol swab.
- Draw **5 mL** of BAC into a mixing syringe.
- Insert through the stopper. Inject slowly down the inside glass wall — not straight onto the powder.
- Gently roll or swirl until clear. Do not shake.
- Wipe the top again. Write today's date on the vial. Refrigerate 36–46°F. Use within 28 days.
- Mix and store in this vial only. Do not transfer.

8. Loading the 0.5 mL syringe — schedule for this vial

U-100 math is the same on a 0.5 mL or 1 mL syringe: 100 units = 1.00 mL, so 50 units = 0.50 mL (a completely full 0.5 mL syringe). Starting draws on this sheet all fit 0.5 mL. The green row is the start.

When	Units (U-100)	mg	mL	0.5 mL syringe	How often
Weeks 1–2 (start here)	50	100	0.50	Draw to 50	Twice per week
Weeks 3+ (if tolerated)	75	150	0.75	Too big — 1 mL syringe or 50+ remainder	Twice per week
Optional advance	100	200	1.00	Too big — 1 mL syringe or 50+ remainder	Twice per week
Optional advance	125	250	1.25	Too big — 1 mL syringe or 50+ remainder	Twice per week

- Draw 5 mL of BAC — this is the most this vial can hold. Mix and store in this vial only.
- 50 units = 0.50 mL = 100 mg. That fill is exactly one full 0.5 mL syringe.
- 125 units = two syringes in one session: 100 + 25 (two nearby sites).
- At 50 units twice per week one mixed vial lasts about 5 weeks.



Fulfillment

- Prep and local delivery: Las Vegas residents only.
- Shipping: dry (lyophilized) vials only — we do not ship mixed liquid.
- Interest orders are confirmed by phone before payment.

Working with the team

- Share a phone number so the team can call.
- Confirm Las Vegas residency for prep and local delivery.
- Complete required documentation with the team.
- If anything on this sheet disagrees with your vial label or COA, stop and call before mixing.

Sources

- TrueHold locked 1000 mg protocol: 5 mL BAC, start 50 units twice weekly (100 mg per shot)
- NAD⁺ biochemistry: electron transport, sirtuins, PARPs (standard cell-biology references)
- TrueHold Standard Starting Protocol locked to the 1000 mg NAD⁺ vial (5 mL BAC, 50-unit start twice weekly).

Protocol note

TrueHold writes these sheets for the milligram vials we hold. Mix volume and unit marks were double-checked so milligrams, milliliters, and U-100 units agree. This is educational research information, not a prescription and not medical advice. The Telegram bot does not dose in chat — read this PDF. For an individualized plan, use /schedule. Research use only.

